

DRUG-SENSITIVE TB (DS-TB): ROUTINE CARE

TB tests changing from 'Xpert Ultra' to 'TB NAAT'
(NAAT = nucleic acid amplification test and includes Xpert as well as newer TB tests).

Assess the patient with DS-TB

Assess	When to assess	Note
Registration	At diagnosis	Ensure patient record completed and captured on TIER.net.
TB contacts	At diagnosis	Advise that all TB contacts ¹ visit the clinic for TB screening and testing or ensure CHW does a home visit for TB screening and testing.
Alcohol/drug use	At diagnosis	In the past year, has patient: 1) drunk ≥ 4 drinks ³ /session, 2) used illegal drugs or 3) misused prescription or over-the-counter medications? If yes to any → 142 .
Symptoms	Every visit	- If respiratory rate ≥ 30, breathless at rest or while talking, prominent use of breathing muscles, drowsy/confused, coughs up ≥ 1 tablespoon fresh blood, neck stiffness, persistent vomiting or new weakness of arm/leg, give urgent attention → 92. - Expect gradual improvement on TB treatment. If symptoms worsen or do not get better, refer to doctor.
Adherence	Every visit	Request patient brings all medication to each visit. Check adherence on the TB card. If poor adherence, manage the patient who interrupts TB treatment → 98 .
Side effects	Every visit	Ask about side effects of treatment → 97 .
Family planning	Every visit	- Encourage patient to avoid pregnancy during treatment, assess patient's contraceptive needs → 154. If pregnant → 159. - Avoid oral contraceptive and subdermal implant² on TB treatment, use instead injectable or IUD <i>plus</i> condoms. No need to change interval between injectable.
Depression	Every visit	In the past month, has patient: 1) felt down, depressed, hopeless or 2) felt little interest or pleasure in doing things? If yes to either → 143 .
Palliative care	If advanced TB disease	If patient is in bed or chair for 50% or more of the day or dependent on others for most care, also give palliative care → 170 .
Weight and BMI	Every visit	- Expect weight gain on treatment and adjust TB treatment dose → 96. If losing weight, refer to doctor/hospital same week. - BMI = weight (kg) $\div$ height (m) $\div$ height (m). If < 18.5, refer for nutritional support.
Glucose	At diagnosis	- If known diabetes, assess glucose control more often → 130. - If not known with diabetes, check glucose → 17.
HIV	At diagnosis and every visit	If > 3 months since last HIV test, test for HIV → 110 . If HIV positive, give routine HIV care and start ART → 111 . If on ART, adjust medication/dosing → 97 .
TB NAAT result	At diagnosis	Register patient as MTB detected, RIF sensitive/ RIF resistant; MTB not detected; Trace. If LAM was used to diagnose TB, review TB NAAT result → 92 .
TB microscopy (smear) ⁴	At diagnosis	If TB NAAT positive at diagnosis, send sputum for smear microscopy. Record smear microscopy result in the patient's file. Register as smear negative or smear positive.
	Week 7	- Do only if smear positive pulmonary TB at diagnosis/registration: - If week 7 smear positive: send 1 sputum for DST, prolong intensive phase for 1 month and manage further as per positive week 7 smear algorithm → 97. - If week 7 smear negative and clinically improving: change to continuation phase for further 4 months.
	Week 23	- Do only if smear positive pulmonary TB at diagnosis. - Use week 23 smear result to decide treatment outcome → 98.
TB culture and DST result	- If sent during diagnostic workup - At 8 weeks: if still smear positive - If HIV positive and TB NAAT negative - At 24 weeks: if still smear positive	- If both TB culture and TB NAAT negative at diagnosis, refer to hospital for further investigation or discuss with experienced TB doctor or specialist. - If MTB (<i>Mycobacterium tuberculosis</i>) on culture, check DST result: - If susceptible to rifampicin and INH, continue treatment. - If resistant to INH only, diagnose INH mono-resistant TB and give routine care → 95. - If resistant to rifampicin, diagnose rifampicin-resistant TB (RR-TB) and give routine care → 98. - If culture contaminated, repeat. If culture shows NTM (Nontuberculous mycobacteria), continue treatment and refer to doctor.
Treatment outcome	At completion of TB treatment	Decide on treatment outcome → 98 .

Advise, counsel and treat the patient with DS-TB **→ 96**.

¹A TB contact refers to a patient who shared an enclosed space (at work, socially, in a hostel, or in a household setting), for ≥ 1 night or for frequent/extended daytime periods, with an adult/adolescent with pulmonary TB ("index patient"), during the 3-month period before the index patient started their TB treatment. ²If patient already has subdermal implant, advise additional non-hormonal method (copper IUD or condoms) until 4 weeks after completing TB treatment. ³One drink is 1 tot of spirits, or 1 small glass (125mL) of wine or 1 can/bottle (330mL) of beer. ⁴Make every effort to obtain sputum. If difficulty, try in early morning or arrange for induced sputum.